# From a working product to a paying partner

MediChain is built and live. The business question is who pays first, what it costs to serve them, and what could go wrong.

## Customers, in order

| Segment | Why they buy | First offer |
|---|---|---|
| Private EMS operators | Responders need allergies and medicines on scene; they handle few records, so they are quick to adopt | Emergency summary and handover, per vehicle or per crew |
| Private clinics and GP groups | One shared record across their sites; patients who can see their own care | Staff portal and patient app, per facility |
| Hospital groups | Emergency department intake and handover; accountability for access | Integration pilot in one emergency department |
| Public facilities | National policy goals; after POPIA authorisation and procurement | Through partners and programmes |

## Revenue model

- **Subscription per facility or vehicle**, priced by active staff, with onboarding and support. Free for patients, always.
- **Implementation fees** for integration with an existing hospital system (FHIR endpoints already exist).
- Prices are hypotheses to set with the first partner; we will not publish numbers we have not tested.

## Cost to serve

| Item | Today | At pilot scale |
|---|---|---|
| Hosting (AWS Cape Town) | One t3.small instance, S3; budget alarm at USD 60 a month | A larger instance and a managed database; cost grows with facilities, not with patients |
| Distribution | Web apps: no app-store fees or review delays | The same, plus an Android app for NFC card emulation |
| Support | Founder | A clinical lead and a support engineer |

## Competitive position

| Alternative | Strength | MediChain's angle |
|---|---|---|
| Paper folders | Familiar; no infrastructure | Available at the point of care; every access recorded |
| Hospital record systems (OpenMRS, Bahmni) | Mature clinical operations | Patient-held emergency summary and patient-visible access; can sit beside them |
| Emergency apps (iER) | Panic alerts, self-reported details | Clinician-maintained data, approved devices, reads shown to the patient |

| | | |
|---|---|---|
| Referral platforms (Vula) | Clinician-to-specialist referrals | Complementary: the patient's own record and emergency access |

Comparisons describe focus, not shortcomings.

## Go-to-market

| Phase | Milestone |
|---|---|
| 1. Pilot (3 months) | One EMS or clinic partner; synthetic training, then an approved pilot of the emergency summary and handover |
| 2. Proof (6 months) | Published pilot measures: seconds to critical information, handover completeness, patient understanding |
| 3. Expansion | Paying facilities in Gauteng and the Western Cape; an Android app; integrations |

## Risks and how we handle them

| Risk | Response |
|---|---|
| Regulatory approval takes time | Start with fictional and consented pilot data; Information Officer and POPIA prior authorisation are tracked as release gates |
| Clinicians will not adopt another system | Begin where the pain is sharpest (the emergency summary), keep each workflow to a few screens, measure adoption per shift |
| Patients lose phones or cards | A card number works by hand; the record is on the server, not the device |
| Single founder | First hires are a clinical lead and a second engineer; the code, tests and runbooks are written so others can take over |
| Connectivity and load-shedding | An offline QR summary; light web apps |

## Team and funding

**Keorapetswe Kgoatlha**, founder and developer, designed and built MediChain end to end (2nd place, Rust Africa Hackathon 2026). Company: Lukau Invasion (Pty) Ltd, 2026/600804/07. Funding sought from grants (for example NYDA and youth-entrepreneurship funds) and pilot revenue, for the first two hires and the regulatory work.

## The ask

A pilot partner, and introductions to funders who back South African health technology.